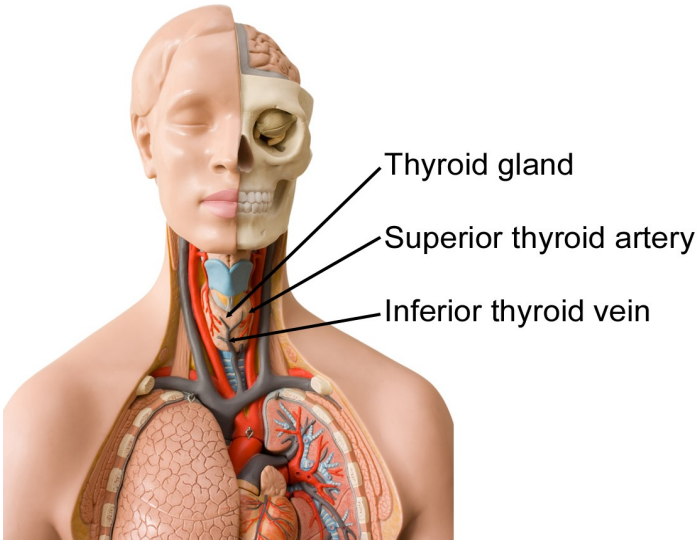
Thyroidectomy is usually performed for malignancy, goiter, thyroid nodules, hyperthyroidism, or Graves' disease. It is traditionally performed through a transverse incision with care to avoid injuring the internal jugular veins on either side of the gland. Blood loss is typically minimal (50-75 mL). Risks of the surgery include hematoma, respiratory difficulty due to inadvertent damage to the recurrent laryngeal nerve (the most common nerve injury associated with this case), and hypocalcemia resulting from removal of the parathyroid glands. If hypocalcemia does occur, it most commonly manifests as tingling of the fingertips, but can present as laryngeal stridor.

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The procedure is generally performed under GETA although it has been performed under local anesthesia in rare circumstances

Patients presenting for this surgery for hyperthyroidism should continue their antithyroid medications preoperatively and may need treatment with beta-blockers to reduce heart rate and blood pressure to a normal state. Avoid medications such as ketamine, pancuronium, and meperidine that could stimulate the sympathetic nervous system. It is also important to establish an adequate depth of anesthesia to prevent a hyperdynamic state in response to surgical incision.

Patients with hypothyroidism may exhibit tracheal deviation from goiter. Opioids and sedatives should be used cautiously because they may also exhibit a decreased ventilatory response to carbon dioxide and hypoxemia. Bradydysrhythmias, diastolic hypertension, ST and T wave changes, and prolongation of the QT interval may be present. Fluid volume expansion should be performed cautiously as diastolic dysfunction and decreased left ventricular compliance may also be present.

Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 672-677.

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